

Emergency Protocol — Imminent Risk & Crisis Situations

Document ID	TG-POL-05	Effective	March 1, 2026
Applies to	All Employees (Therapists, Counselors, Coaches)	Owner	Clinical & Compliance Team

At a Glance

Imminent risk	Active suicidal ideation with plan/means/intent, intent to harm another, disclosure of ongoing abuse of a child/vulnerable adult, or a live medical emergency.
First move	Stay on the call, ask directly and calmly, do not end the session abruptly.
Escalate to	On-call Clinical Supervisor via "Escalate Now" (24/7, first contact for every role) → local emergency services if immediate danger and supervisor unreachable within 2 minutes.
Every role	Coaches and Counselors: recognize risk, stay engaged, escalate — never manage a crisis alone. Therapists: may assess, but must still loop in the supervisor.

1. Purpose

This policy governs how every Togethr employee — regardless of role or license — must respond when a client discloses imminent risk to themselves or others during a session. This protocol overrides all standard scope-of-practice and referral procedures. Every employee, including Coaches, is expected to know this protocol and act on it immediately.

2. What Counts as Imminent Risk

- Active suicidal ideation with a stated plan, means, or intent
- Statements suggesting an intent to harm another specific person
- Disclosure of ongoing abuse or neglect of a child or vulnerable adult
- A medical emergency occurring during the session (e.g., client appears to be in acute physical distress)
- Any disclosure that leaves the employee genuinely uncertain whether the client is safe right now

If there is any doubt about whether a situation qualifies, treat it as imminent risk. It is always better to over-escalate than under-escalate.

3. Immediate Steps During a Live Session

- Stay on the call with the client. Do not end the session abruptly.
- Ask directly and calmly: "Are you thinking about hurting yourself right now?" Direct questions do not increase risk — avoiding the question does not make it safer.
- If the client confirms a plan, means, or intent, activate the Escalation Chain below while remaining present with the client.
- If you are a Coach or Counselor, use the in-app "Escalate Now" button to loop in the on-call clinical supervisor in real time — you are not expected to manage a crisis alone.
- Do not promise confidentiality in this moment. Be honest that you need to involve others to keep the client safe.

4. Escalation Chain

- On-call Clinical Supervisor — reachable via the in-app "Escalate Now" button, 24/7. This is the first point of contact for every role.
- Local emergency services (911 or the equivalent in the client's location) — contact directly if there is an active, immediate danger to life and the on-call supervisor is not reachable within 2 minutes
- 988 Suicide & Crisis Lifeline — may be offered to the client as an additional resource, but does not replace direct escalation by the employee
- Mandatory reporting hotline (for suspected child or vulnerable adult abuse) — the on-call supervisor will confirm reporting obligations and, where required under Illinois law, ensure a report is filed within the legally required window

5. After the Session

- Document the disclosure, your response, and the escalation outcome within 1 hour of the session ending
- Debrief with the on-call clinical supervisor the same day
- Follow up directly with the client within 24 hours
- A safety plan should be created or updated with the client's ongoing provider as appropriate

6. Special Notes by Role

Coaches and Counselors are never expected to conduct a full risk assessment on their own — their responsibility is to recognize risk, keep the client engaged, and escalate immediately to a licensed

clinician. Therapists may conduct an initial risk assessment using their clinical judgment but must still loop in the on-call supervisor for any confirmed imminent risk; a Therapist's independent judgment does not replace the escalation chain.

7. Legal & Mandatory Reporting Notes

Illinois law requires mandatory reporting of suspected child abuse or neglect by licensed mental health professionals. Togethr employees in any role are treated as mandatory reporters for the purposes of this policy, regardless of license status. Duty-to-warn obligations (notifying a specific, identifiable potential victim of harm) follow Illinois's Mental Health and Developmental Disabilities Confidentiality Act and are coordinated through the on-call clinical supervisor — individual employees should not attempt to make this determination alone.

8. Example Scenarios

- A client says, "Sometimes I wonder if everyone would be better off without me," but denies any plan or intent when asked directly. This still warrants a direct safety conversation and should be discussed with the on-call supervisor the same day, even if it does not rise to the level of an active plan.
- A client states they have a specific plan and access to means to end their life tonight. This is textbook imminent risk — escalate via "Escalate Now" immediately while remaining on the call.
- A client mentions their partner "gets physical" during arguments, but there is no indication of immediate danger right now. This is not necessarily an in-the-moment emergency, but should be discussed with a supervisor about appropriate next steps, which may include referral resources (TG-POL-04) and safety planning.
- A client becomes unresponsive or shows signs of a medical emergency on a video call. Treat this as an active emergency — attempt to get the client's location if known and contact local emergency services immediately.

9. Frequently Asked Questions

Q: What if I'm not sure whether something counts as imminent risk?

A: Treat it as imminent risk and escalate. There is no penalty for escalating a situation that turns out not to require it — there is significant risk in under-escalating.

Q: Can a Coach be expected to ask directly about suicidal thoughts?

A: Yes. Every employee, regardless of role, is expected to ask directly if there is any indication of risk. Direct questions save lives; avoiding the question does not protect the client.

Q: What happens after I escalate to the on-call supervisor?

A: The supervisor will guide the remainder of the session in real time if needed, coordinate any external contact (emergency services, mandatory reporting), and lead the same-day debrief.

10. Related Policies

This is the single most important policy every employee must know regardless of role. It should be read alongside Referral Protocol (TG-POL-04) and the scope-of-practice policy for your role (TG-POL-01, TG-POL-02, or TG-POL-03).